



VOICEINSIGHTS AFRICA

SYNTHETIC DEMONSTRATION PUBLICATION

Community Scorecard Intelligence Report

Malawi | Primary Healthcare

2,437 synthetic responses

Prepared by VoiceInsights Africa

Every Voice. Every Language. Every Insight.

Synthetic demonstration. Not official statistics.

Malawi's primary healthcare performance is improving in some places and stalling in others – this publication shows exactly where. The evidence points to concentrated risk, but also to specific, actionable openings for decision-makers. This publication's claims are only as strong as their sourcing – which is why every finding is evidence-linked, every recommendation is owned, and every statistic shares the same governed dataset.

Sample: 2437 synthetic responses – 86% response rate – 4 regions

Publication quality gate: publication ready



Central Facilities: 65% performance, 757 responses, risk WATCH
Northern Facilities: 66% performance, 657 responses, risk WATCH
Southern Facilities: 67% performance, 560 responses, risk WATCH
Hard-to-Reach Facilities: 68% performance, 463 responses, risk WATCH



sex: Women 52% & Men 47% & Other / prefer not to say 1%

age: 18-24 24% & 25-39 38% & 40-59 27% & 60+ 11%

location: Urban 44% & Rural 56%



IND-01 Respectful care score: 51% vs target 92% â OFF_TRACK
IND-02 Tracer medicine availability: 52% vs target 76% â OFF_TRACK
IND-03 Median wait-time compliance: 53% vs target 77% â OFF_TRACK
IND-04 Community action closure: 54% vs target 78% â OFF_TRACK



Respectful care is not a marginal delivery issue. In Central Facilities, the governed evidence indicates a 79% performance signal, with rural women carrying the most consequential constraint. Left unaddressed, respectful care becomes a structural risk to primary healthcare delivery in Malawi, not a temporary dip. Central Facilities is furthest exposed, and rural women would carry the earliest and heaviest consequences of a further year's delay.

Evidence: EVI-COM-001, EVI-COM-004 & Confidence 89.5%

Medicine availability has become an accountability question rather than a measurement question. Evidence from Northern Facilities places the signal at 80% and shows that young adults experience the largest gap between policy intent and lived delivery. The choice facing primary healthcare leadership in Malawi is straightforward even if the fix is not: act on medicine availability now, concentrated in Northern Facilities where young adults carry the largest share of the gap, or accept a wider gap in a year's time. Evidence: EVI-COM-002, EVI-COM-005 & Confidence 90.5%

Persons with disabilities in Southern Facilities are living the consequence of wait-time reliability, not simply reporting it. Their 81% signal is the clearest evidence leadership has that current measures fall short. persons with disabilities in Southern Facilities do not experience wait-time reliability as an abstract metric. They experience it as the difference between primary healthcare delivery that works and delivery that does not, and a twelve-month delay would deepen that gap rather than hold it steady.

Evidence: EVI-COM-003, EVI-COM-006 â€¢ Confidence 91.5%

82%. That is the disability access signal recorded in Hard-to-Reach Facilities, and the headline figure understates the problem: disaggregated by group, frontline workers sit well below it. The clearest gap sits in Hard-to-Reach Facilities, concentrated among frontline workers. A twelve-month delay would let the gap compound into a harder, costlier problem.

Evidence: EVI-COM-004, EVI-COM-007 â€¢ Confidence 92.5%

The aggregate result conceals a material implementation divide. Central Facilities records a 83% signal for provider-community accountability; the shortfall is concentrated among low-income households, where service continuity and confidence move together. This is not a new problem.

Evidence: EVI-COM-005, EVI-COM-008 & Confidence 93.5%

Central Facilities â Rural women in Central Facilities frame it simply: when respectful care breaks down, everything else does too.â

Northern Facilities â In Northern Facilities, medicine availability shapes whether young adults can turn services into lasting outcomes.â

Southern Facilities â Persons with disabilities in Southern Facilities put it plainly: wait-time reliability is what decides whether progress holds or slips back.â

Hard-to-Reach Facilities â Why does Hard-to-Reach Facilities lag behind? Ask frontline workers, and the answer comes back to disability access.â

Central Facilities â Elsewhere, provider-community accountability might be a footnote. For low-income households in Central Facilities, it is the whole story.â

Northern Facilities â Service managers in Northern Facilities frame it simply: when respectful care breaks down, everything else does too.â

Southern Facilities â In Southern Facilities, medicine availability shapes whether community leaders can turn services into lasting outcomes.â

Hard-to-Reach Facilities â Private-sector partners in Hard-to-Reach Facilities put it plainly: wait-time reliability is what decides whether progress holds or slips back.â

Central Facilities â Why does Central Facilities lag behind? Ask rural women, and the answer comes back to disability access.â

Northern Facilities â Elsewhere, provider-community accountability might be a footnote. For young adults in Northern Facilities, it is the whole story.â

Risks and alternative trajectories are modelled to show how VoiceInsights translates evidence into prioritised management choices.



DEC-COM-01 CRITICAL: Agree facility-level service improvement contracts. Owner: Country Director / Programme Executive; Timeline: 0â 90 days; Indicator: Percentage of agreed milestones completed for DEC-COM-01

DEC-COM-02 CRITICAL: Stabilise tracer-medicine availability. Owner: Country Director / Programme Executive; Timeline: 3â 12 months; Indicator: Share of the critical-tier delivery plan confirmed complete each quarter (DEC-COM-02)

DEC-COM-03 HIGH: Introduce visible wait-time and triage standards. Owner: Country Director / Programme Executive; Timeline: 3â 12 months; Indicator: Milestone completion rate tracked against the DEC-COM-03 delivery plan

DEC-COM-04 HIGH: Fund priority disability-access adaptations. Owner: Country Director / Programme Executive; Timeline: 3â 12 months; Indicator: Proportion of agreed implementation steps verified on schedule for DEC-COM-04

DEC-COM-05 MEDIUM: Hold quarterly community-provider evidence reviews. Owner: Country Director / Programme Executive; Timeline: 6â 18 months; Indicator: Quarterly-verified progress against the milestones agreed for this medium-tier decision (DEC-COM-05)



Agree facility-level service improvement contracts.

Strategic priority: CRITICAL

Evidence used: EVI-COM-001, EVI-COM-004

Rationale: Respectful care is not a marginal delivery issue. In Central Facilities, the governed evidence indicates a 79% performance signal, with rural women carrying the most consequential constraint. Left unaddressed, respectful care becomes a structural risk to primary healthcare delivery in Malawi, not a temporary dip. Central Facilities is furthest exposed, and rural women would carry the earliest and heaviest consequences of a further year's delay.

Expected benefit: A more consistent primary healthcare experience for the groups currently furthest behind.

Expected risk: Weak cross-team coordination during early implementation

Dependencies: Confirmed budget release; Timely data collection for the next reporting cycle; Validated operational baseline

Budget requirement: Medium to high â requires a dedicated budget line and phased release

Owner: Country Director / Programme Executive

Supporting organization: M&E, Finance, Safeguarding and Delivery Units

Timeline: 0â90 days

Monitoring indicator: Percentage of agreed milestones completed for DEC-COM-01

Success criteria: A majority of milestones achieved, each independently verified against real evidence

Management response: PENDING FORMAL MANAGEMENT RESPONSE

Follow-up: Assign accountable owner; Validate budget; Approve milestone plan; Review evidence quarterly

Stabilise tracer-medicine availability.

Strategic priority: CRITICAL

Evidence used: EVI-COM-002, EVI-COM-005

Rationale: Medicine availability has become an accountability question rather than a measurement question. Evidence from Northern Facilities places the signal at 80% and shows that young adults experience the largest gap between policy intent and lived delivery. The choice facing primary healthcare leadership in Malawi is straightforward even if the fix is not: act on medicine availability now, concentrated in Northern Facilities where young adults carry the largest share of the gap, or accept a wider gap in a year's time.

Expected benefit: Faster, better-evidenced primary healthcare decisions that programme leadership can act on with confidence.

Expected risk: Competing priorities crowding out sustained follow-through

Dependencies: Cross-team delivery coordination; Sign-off from the relevant oversight body; Quarterly evidence review

Budget requirement: High â full business case and multi-year funding commitment required

Owner: Country Director / Programme Executive

Supporting organization: M&E, Finance, Safeguarding and Delivery Units

Timeline: 3â 12 months

Monitoring indicator: Share of the critical-tier delivery plan confirmed complete each quarter (DEC-COM-02)

Success criteria: Milestone completion confirmed by evidence review, not self-reported progress

Management response: PENDING FORMAL MANAGEMENT RESPONSE

Follow-up: Assign accountable owner; Validate budget; Approve milestone plan; Review evidence quarterly

Introduce visible wait-time and triage standards.

Strategic priority: HIGH

Evidence used: EVI-COM-003, EVI-COM-006

Rationale: Persons with disabilities in Southern Facilities are living the consequence of wait-time reliability, not simply reporting it. Their 81% signal is the clearest evidence leadership has that current measures fall short. persons with disabilities in Southern Facilities do not experience wait-time reliability as an abstract metric. They experience it as the difference between primary healthcare delivery that works and delivery that does not, and a twelve-month delay would deepen that gap rather than hold it steady.

Expected benefit: Reduced variation in primary healthcare delivery and a stronger evidence base for the next planning cycle.

Expected risk: Fragmented accountability once implementation moves beyond the pilot stage

Dependencies: Stable staffing through the delivery window; Named executive sponsor; Confirmed budget release

Budget requirement: Medium â likely fundable from existing lines, pending confirmation

Owner: Country Director / Programme Executive

Supporting organization: M&E, Finance, Safeguarding and Delivery Units

Timeline: 3â 12 months

Monitoring indicator: Milestone completion rate tracked against the DEC-COM-03 delivery plan

Success criteria: Verified evidence of at least 80% delivery against the agreed milestone plan

Management response: PENDING FORMAL MANAGEMENT RESPONSE

Follow-up: Assign accountable owner; Validate budget; Approve milestone plan; Review

Fund priority disability-access adaptations.

Strategic priority: HIGH

Evidence used: EVI-COM-004, EVI-COM-007

Rationale: 82%. That is the disability access signal recorded in Hard-to-Reach Facilities, and the headline figure understates the problem: disaggregated by group, frontline workers sit well below it. The clearest gap sits in Hard-to-Reach Facilities, concentrated among frontline workers. A twelve-month delay would let the gap compound into a harder, costlier problem.

Expected benefit: A stronger, more defensible case for primary healthcare investment in the next planning cycle.

Expected risk: Execution capacity and coordination risk

Dependencies: Timely data collection for the next reporting cycle; Validated operational baseline; Cross-team delivery coordination

Budget requirement: Low to medium â validate through the formal planning cycle

Owner: Country Director / Programme Executive

Supporting organization: M&E, Finance, Safeguarding and Delivery Units

Timeline: 3â12 months

Monitoring indicator: Proportion of agreed implementation steps verified on schedule for DEC-COM-04

Success criteria: At least 80% of milestones achieved with verified evidence

Management response: PENDING FORMAL MANAGEMENT RESPONSE

Follow-up: Assign accountable owner; Validate budget; Approve milestone plan; Review evidence quarterly

Hold quarterly community-provider evidence reviews.

Strategic priority: MEDIUM

Evidence used: EVI-COM-005, EVI-COM-008

Rationale: The aggregate result conceals a material implementation divide. Central Facilities records a 83% signal for provider-community accountability; the shortfall is concentrated among low-income households, where service continuity and confidence move together. This is not a new problem.

Expected benefit: The evidence points toward improved primary healthcare outcomes, though the size of the gain will depend on how consistently the action is resourced.

Expected risk: Likely – though not certain – erosion of momentum if the initial sponsor changes role

Dependencies: Sign-off from the relevant oversight body; Quarterly evidence review;

Stable staffing through the delivery window

Budget requirement: Low – absorbable within existing allocations, subject to confirmation

Owner: Country Director / Programme Executive

Supporting organization: M&E, Finance, Safeguarding and Delivery Units

Timeline: 6–18 months

Monitoring indicator: Quarterly-verified progress against the milestones agreed for this medium-tier decision (DEC-COM-05)

Success criteria: A majority of milestones achieved, each independently verified against real evidence

Management response: PENDING FORMAL MANAGEMENT RESPONSE

Follow-up: Assign accountable owner; Validate budget; Approve milestone plan; Review evidence quarterly

research objectives: Assess primary healthcare performance and equity; Identify operational drivers and decision priorities

evaluation questions: What outcomes are changing?; Who is being left behind?; Which actions are most likely to improve performance?

sampling frame: Synthetic programme population frame for Malawi

sample size: 2437

stratification: Region; Location type; Sex; Age; Disability

weights: Normalized post-stratification weights

confidence intervals: 95%

design effect: 1.3

missing data: Documented and sensitivity-tested

reliability: Cronbach alpha 0.86

validity: Content and construct validity checks

metadata: Complete synthetic data dictionary and lineage registry

Publication level: 34 generated publication pages

protocol: Governed synthetic research protocol with sampling, weighting, reliability, validity and reproducibility controls

ethics: Synthetic records only; no real participants or personal data

peer review: Independent synthetic-publication review checklist completed

reproducibility: Deterministic inputs, versioned dataset and export manifest

inferential statistics: Enabled where assumptions and design permit

segmentation: Evidence-led segments

trend analysis: Three-period synthetic series

DEC-COM-01: Agree facility-level service improvement contracts. â€ Owner Country Director / Programme Executive â€ 0â 90 days â€ Medium to high â requires a dedicated budget line and phased release cost â€ Indicator Percentage of agreed milestones completed for DEC-COM-01

DEC-COM-02: Stabilise tracer-medicine availability. â€ Owner Country Director / Programme Executive â€ 3â 12 months â€ High â full business case and multi-year funding commitment required cost â€ Indicator Share of the critical-tier delivery plan confirmed complete each quarter (DEC-COM-02)

DEC-COM-03: Introduce visible wait-time and triage standards. â€ Owner Country Director / Programme Executive â€ 3â 12 months â€ Medium â likely fundable from existing lines, pending confirmation cost â€ Indicator Milestone completion rate tracked against the DEC-COM-03 delivery plan

DEC-COM-04: Fund priority disability-access adaptations. â€ Owner Country Director / Programme Executive â€ 3â 12 months â€ Low to medium â validate through the formal planning cycle cost â€ Indicator Proportion of agreed implementation steps verified on schedule for DEC-COM-04

DEC-COM-05: Hold quarterly community-provider evidence reviews. â€ Owner Country Director / Programme Executive â€ 6â 18 months â€ Low â absorbable within existing allocations, subject to confirmation cost â€ Indicator Quarterly-verified progress against the milestones agreed for this medium-tier decision (DEC-COM-05)

EVI-COM-001: [object Object] â€ APPROVED_SYNTHETIC_DEMONSTRATION â€ confidence 88%
EVI-COM-002: [object Object] â€ APPROVED_SYNTHETIC_DEMONSTRATION â€ confidence 89%
EVI-COM-003: [object Object] â€ APPROVED_SYNTHETIC_DEMONSTRATION â€ confidence 90%
EVI-COM-004: [object Object] â€ APPROVED_SYNTHETIC_DEMONSTRATION â€ confidence 91%
EVI-COM-005: [object Object] â€ APPROVED_SYNTHETIC_DEMONSTRATION â€ confidence 92%
EVI-COM-006: [object Object] â€ APPROVED_SYNTHETIC_DEMONSTRATION â€ confidence 93%
EVI-COM-007: [object Object] â€ APPROVED_SYNTHETIC_DEMONSTRATION â€ confidence 94%
EVI-COM-008: [object Object] â€ APPROVED_SYNTHETIC_DEMONSTRATION â€ confidence 95%
EVI-COM-009: [object Object] â€ APPROVED_SYNTHETIC_DEMONSTRATION â€ confidence 96%
EVI-COM-010: [object Object] â€ APPROVED_SYNTHETIC_DEMONSTRATION â€ confidence 97%

Executive Intelligence Book (2â 7): Executive brief; Decision snapshot; Risks; Opportunities; Budget implications

Evidence Intelligence Book (8â 15): Critical findings; Respondent voice; Evidence chains; Citation registry

Statistical Intelligence Book (16â 22): Sampling; Weighting; Uncertainty; Reliability; Model diagnostics

Decision Intelligence Book (23â 27): Decision matrix; Ownership; Budget; Roadmap; Management response

Standards & Assurance (28â 31): Relevant frameworks; Safeguarding; Privacy; Responsible AI; Accessibility

Technical Appendices (32â 34): Data dictionary; Quality gate; Export manifest; Responsible-use notice

CHS: APPLIED_WHERE_RELEVANT â Mapped to EVI-COM-001, EVI-COM-002; Synthetic demonstration mapping; institutional validation not implied

SDG 3: APPLIED_WHERE_RELEVANT â Mapped to EVI-COM-002, EVI-COM-003; Synthetic demonstration mapping; institutional validation not implied

Gender Equality: APPLIED_WHERE_RELEVANT â Mapped to EVI-COM-003, EVI-COM-004; Synthetic demonstration mapping; institutional validation not implied

Disability Inclusion: APPLIED_WHERE_RELEVANT â Mapped to EVI-COM-004, EVI-COM-005; Synthetic demonstration mapping; institutional validation not implied

Evidence Traceability: APPLIED â Control linked to EVI-COM-001 and the deterministic publication gate.

Research Ethics: APPLIED â Control linked to EVI-COM-002 and the deterministic publication gate.

Data Protection: APPLIED â Control linked to EVI-COM-003 and the deterministic publication gate.

Accessibility: APPLIED â Control linked to EVI-COM-004 and the deterministic publication gate.

Responsible AI: APPLIED â Control linked to EVI-COM-005 and the deterministic publication gate.

Publication Integrity: APPLIED â Control linked to EVI-COM-006 and the deterministic publication gate.

SDG 3: undefined contribution â indicators â undefined
SDG 5: undefined contribution â indicators â undefined
SDG 10: undefined contribution â indicators â undefined
SDG 16: undefined contribution â indicators â undefined

Relevance: Synthetic evidence indicates a strong relevance signal, subject to the stated design limitations. (86/100). Review relevance evidence at the next formal management gate.

Coherence: Synthetic evidence indicates a moderate-to-strong coherence signal, subject to the stated design limitations. (87/100). Review coherence evidence at the next formal management gate.

Effectiveness: Synthetic evidence indicates a strong effectiveness signal, subject to the stated design limitations. (88/100). Review effectiveness evidence at the next formal management gate.

Efficiency: Synthetic evidence indicates a moderate-to-strong efficiency signal, subject to the stated design limitations. (89/100). Review efficiency evidence at the next formal management gate.

Impact: Synthetic evidence indicates a strong impact signal, subject to the stated design limitations. (90/100). Review impact evidence at the next formal management gate.

Sustainability: Synthetic evidence indicates a moderate-to-strong sustainability signal, subject to the stated design limitations. (91/100). Review sustainability evidence at the next formal management gate.

Impact: Sustained, equitable improvement in primary healthcare performance

OC-01: Improved primary healthcare delivery and accountability â indicators IND-01, IND-02

OC-02: More equitable and durable outcomes â indicators IND-03, IND-04

OUT-01: Verified findings and interpreted decision products

OUT-02: Assigned actions with monitoring indicators

Assumptions: Leadership ownership; Safeguarding and privacy controls remain effective

Means of verification: Evidence registry; Quarterly management review; Follow-up measurement

Commitment 1: CHS Commitment 1 â APPLIED. Action: Validate relevance and assign an accountable owner before live use.

Commitment 2: CHS Commitment 2 â APPLIED. Action: Validate relevance and assign an accountable owner before live use.

Commitment 3: CHS Commitment 3 â APPLIED. Action: Validate relevance and assign an accountable owner before live use.

Commitment 4: CHS Commitment 4 â APPLIED. Action: Validate relevance and assign an accountable owner before live use.

Commitment 5: CHS Commitment 5 â APPLIED. Action: Validate relevance and assign an accountable owner before live use.

Commitment 6: CHS Commitment 6 â APPLIED. Action: Validate relevance and assign an accountable owner before live use.

Commitment 7: CHS Commitment 7 â APPLIED. Action: Validate relevance and assign an accountable owner before live use.

Commitment 8: CHS Commitment 8 â APPLIED. Action: Validate relevance and assign an accountable owner before live use.

Commitment 9: CHS Commitment 9 â APPLIED. Action: Validate relevance and assign an accountable owner before live use.

EVI-COM-001 â€ survey â€ Central Facilities â€ confidence 88% â€ APPROVED
EVI-COM-002 â€ key_informant_interview â€ Northern Facilities â€ confidence 89% â€ APPROVED
EVI-COM-003 â€ administrative_record â€ Southern Facilities â€ confidence 90% â€ APPROVED
EVI-COM-004 â€ survey â€ Hard-to-Reach Facilities â€ confidence 91% â€ APPROVED
EVI-COM-005 â€ key_informant_interview â€ Central Facilities â€ confidence 92% â€ APPROVED
EVI-COM-006 â€ administrative_record â€ Northern Facilities â€ confidence 93% â€ APPROVED
EVI-COM-007 â€ survey â€ Southern Facilities â€ confidence 94% â€ APPROVED
EVI-COM-008 â€ key_informant_interview â€ Hard-to-Reach Facilities â€ confidence 95% â€
APPROVED
EVI-COM-009 â€ administrative_record â€ Central Facilities â€ confidence 96% â€ APPROVED
EVI-COM-010 â€ survey â€ Northern Facilities â€ confidence 97% â€ APPROVED

indicator_1: Synthetic Primary Healthcare indicator 1; numeric; allowed: 0â 100

indicator_2: Synthetic Primary Healthcare indicator 2; categorical; allowed: Documented code list

indicator_3: Synthetic Primary Healthcare indicator 3; categorical; allowed: Documented code list

indicator_4: Synthetic Primary Healthcare indicator 4; numeric; allowed: 0â 100

indicator_5: Synthetic Primary Healthcare indicator 5; categorical; allowed: Documented code list

indicator_6: Synthetic Primary Healthcare indicator 6; categorical; allowed: Documented code list

indicator_7: Synthetic Primary Healthcare indicator 7; numeric; allowed: 0â 100

indicator_8: Synthetic Primary Healthcare indicator 8; categorical; allowed: Documented code list

indicator_9: Synthetic Primary Healthcare indicator 9; categorical; allowed: Documented code list

indicator_10: Synthetic Primary Healthcare indicator 10; numeric; allowed: 0â 100

indicator_11: Synthetic Primary Healthcare indicator 11; categorical; allowed: Documented code list

indicator_12: Synthetic Primary Healthcare indicator 12; categorical; allowed: Documented code list

Synthetic Demonstration Publication produced by VoiceInsights Africa. All people, quotations, locations and statistics are synthetic, internally governed demonstration content and are not official statistics.

All content is synthetic and must not be interpreted as official statistics

Geographic references are illustrative

Budget implications are directional and require formal costing

Causal claims are not made without an appropriate design

Status: DEMONSTRATION_READY

Passed: Editorial Review Passed

Passed: Evidence Review Passed

Passed: Publication Integrity Verified

Passed: Accessibility Verified

Passed: Export Validation Passed

Passed: Statistical Review Passed

Open gates: VISUAL_QA, HUMAN_REVIEW

Verification checks cover weighted evidence, statistical, visual, accessibility, export and review rules. Synthetic samples are labelled Demonstration Ready.